Patient Care Goals

1. Provide equitable and tailored care to patients with functional needs to address their specific physical, sensory, cognitive, or mental health requirements.
2. Ensure effective communication and accommodation to facilitate accurate assessment, treatment, and safe disposition.
3. Promote patient safety and dignity while respecting autonomy and cultural considerations.

Patient Inclusion and Exclusion Criteria

Inclusion Criteria

- Patients with functional needs, including:
    - Physical disabilities (e.g., mobility impairments, limb amputations).
    - Sensory impairments (e.g., visual or auditory disabilities).
    - Cognitive or intellectual disabilities (e.g., developmental delays, dementia).
    - Mental health conditions (e.g., schizophrenia, severe anxiety).
- Patients requiring modified assessment or treatment approaches due to their functional needs.

Exclusion Criteria

- Patients without identified functional needs impacting prehospital care delivery.
- Patients whose primary complaint is unrelated to functional needs and requires standard care protocols (e.g., isolated medical emergencies without disability considerations).

Key Patient Assessment Points

1. **Identification of Functional Needs**
    - Recognize functional needs through patient self-report, family/bystander input, medical alert bracelets, or visible assistive devices (e.g., wheelchairs, hearing aids).
    - Note specific impairments (e.g., mobility, communication, or cognitive) affecting care delivery.
2. **Communication Barriers**
    - Assess for language, sensory, or cognitive barriers (e.g., non-verbal status, deafness, or expressive aphasia).
    - Identify patient's preferred communication methods (e.g., sign language, written notes, or lip reading).
3. **Assistive Device Assessment**
    - Evaluate the presence and functionality of assistive devices (e.g., prostheses, white canes, or tracheostomy speaking valves).
    - Confirm whether devices need to accompany the patient or require special handling.

Patient Management & Interventions

1. **Primary Survey**
    - Ensure airway patency, adequate breathing, and circulation, as per Universal Care CPG adapting techniques to accommodate functional needs (e.g., modified jaw thrust for mobility-impaired patients).
    - Control catastrophic external bleeding if present, using accessible methods for patients with sensory impairments.
    - Assess AVPU (Alert, Verbal, Painful, Unresponsive), adjusting for communication or cognitive impairments.
2. **Secondary Survey**
    - Conduct a focused examination as per Universal Care CPG adapting to the patient's functional needs (e.g., simplified questions for cognitive impairments).
    - Obtain OPQRST and SAMPLE history using alternative communication methods (e.g., written or visual aids) if needed.
    - Reassess vital signs, accommodating sensory or physical limitations:
        - Red Priority (unstable, e.g., critical condition): Every 5 minutes.
        - Yellow/Green Priority (stable, e.g., minor complaint): Every 15 minutes.
3. **Consider Red Flags**
    - Inability to communicate critical symptoms due to sensory or cognitive impairments, risking under-triage.

- Unstable conditions (e.g., hypoxia, hypotension) complicated by functional needs, requiring urgent transport.
- Absence of a caregiver or assistive devices, compromising safe disposition.
- Service animal distress or uncontrolled behavior, necessitating alternative transport plans.
- Suspected abuse or neglect in vulnerable patients with functional needs, requiring reporting.

4. **Functional Needs-Specific Interventions**
    - Communication Support:
        - Use translation cards, sign language, or telephone interpreters for language or sensory barriers.
        - Simplify explanations for cognitive impairments, ensuring patient understanding.
    - Assistive Device Management:
        - Ensure assistive devices (e.g., hearing aids, wheelchairs) accompany the patient or are safely stored.
        - Document device details (e.g., type, functionality) for handover.
    - Caregiver Involvement:
        - Engage family or caregivers to assist with history, communication, or calming the patient.
        - Confirm caregiver's role in disposition planning (e.g., treatment at scene or transport).

Patient Safety Considerations

- Elderly Patients (>65 years)
    - Functional needs (e.g., dementia, hearing loss) may complicate assessment; involve caregivers and use simplified communication.
    - Ensure safe disposition with adequate support (e.g., caregiver, home safety) to prevent harm post-care.
- Pediatric Patients (<13 years)
    - Use age-appropriate communication aids (e.g., visual cues, parent involvement) for developmental or sensory impairments.
    - Involve a parent or guardian to confirm understanding and ensure safe follow-up care.
- Communication Barriers
    - Use multiple communication methods (e.g., written, visual, or interpreter services) to ensure accurate history and consent.
    - Avoid assumptions about capacity based on disability; assess thoroughly and document findings.
- Abuse and Neglect
    - Be vigilant for signs of abuse or neglect in vulnerable patients (e.g., unexplained injuries, caregiver coercion).

Documentation

- Complete a comprehensive electronic Patient Care Record (ePCR), capturing the following elements to support continuity of care. Including:
    - Identification: Patient's full name, age, gender, and any noted functional needs.
    - Mechanism / Medical Complaint: Reason for EMS call and any disability-related barriers to communication or assessment.
    - Injuries / Information: Sensory, mobility, or cognitive impairments noted; assistive devices in use; caregiver involvement.
    - Signs: Vital signs, AVPU or GCS score, signs of instability or distress, red flags such as absence of caregiver or neglected appearance.
    - Treatment: Communication methods used, involvement of caregivers, type and handling of assistive devices, modified assessments or repositioning.
    - Allergies: Medication or environmental allergies reported by patient or caregiver.
    - Medications: Current prescriptions, especially psychiatric, seizure, or mobility-related medications.
    - Background: Past medical history including cognitive, physical, or sensory disabilities.
    - Other: Coordination needs at destination, risks identified (e.g., suspected abuse or inability to self-care).

Key Performance Indicators

1. Identify and document functional needs in the patient care report.

Clinical Insights & Additional Guidance

- Common Functional Needs
    - Mobility impairments (e.g., wheelchair users, amputees).
    - Sensory impairments (e.g., blindness, deafness).
    - Cognitive or mental health conditions (e.g., autism, schizophrenia).
    - Chronic conditions requiring assistive devices (e.g., tracheostomies, dialysis shunts).
- Possible Alternative Causes
    - Misinterpretation of symptoms due to communication barriers (e.g., non-verbal patient appearing unresponsive).
    - Underlying medical emergencies (e.g., hypoglycemia, stroke) masked by functional needs, requiring careful assessment.

References

1. Frontera, W. R., DeLisa, J. A., Basford, J., Bockenek, W. L., Chae, J. and Robinson, L. R., 2020. *DeLisa's physical medicine & rehabilitation: Principles and practice*. Philadelphia: Wolters Kluwer.
2. High, K. P., 2019. 'Using functional assessment to define therapeutic goals and treatment', *Journal of the American Geriatrics Society*, 67(9), pp. 1782–1789. doi: 10.1111/jgs.16022.
3. Lowe, S., Rodriguez, A. and Glynn, L., 2013. 'New technology–based functional assessment tools should avoid the weaknesses and proliferation of manual functional assessments', *Journal of Clinical Epidemiology*, 66(6), pp. 619–632. doi: 10.1016/j.jclinepi.2012.12.003.
4. National Association of EMS Officials (NAEMSO), 2022. *Functional needs planning guide for emergency medical services*. Available at: https://naemso.org (Accessed: 20 June 2025).
5. Ring, H., 2007. 'Functional assessment in rehabilitation medicine: clinical applications', *Eura Medicophys*, 43(4), pp. 551–555.